

FAX

Center of Revitalizing Psychiatry
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TO:

Name: Carhall Lawfirm

Fax Number: (941) 914-9629

of Pages: 23

(including cover sheet)

FROM:

Name: Center of Revitalizing Psychiatry

Fax Number: (941) 677-3367

Subject: Medical records request

Message:

Please see the attached medical record request for Michelle, Kohn. The client has paid for the request as of 02/19/24. this is for invoice Number 1000173MC If you have any questions please feel free to contact our office.

Kind Regards,

Meredith Cornett

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 275089689 Encounter Date: 02/27/2023

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that she understands the limitations of telehealth and consents to proceed with treatment through this mode, zoom.

This is a 35 year old woman with a history of ADHD inattentive type, depression and generalized anxiety disorder. Medication Management visit.

Today Michelle reports "I am feeling ok. I do feel better, I took Sertraline for couple weeks, but I did not like how it made me feel so I stopped. However, my depression and anxiety are pretty good now".

DEPRESSION: "improved": "Feelings of sadness/unhappiness, Irritability or frustration, Loss of interest or pleasure in activities, Indecisiveness, distractibility and decreased concentration, Fatigue, tiredness and loss of energy, Feelings of worthlessness, Feelings of guilt, lacking motivation, crying".

ADHD: "so so".

ANXIETY: "greatly improved".

COMPLIANCE WITH MEDICATIONS: Yes. SIDE EFFECTS: Denies.

Sleep: "good, my normal at least 7-8 hours per night, no issues with sleep".

Appetite: "good"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

History Of Present Illness:

She lives with boyfriend and daughter.

Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

Florida PMP: overdose risk score 0/999. 1/9/2023 Methylphenidate 20mg 75/30. 11/8/22 Adderall 20mg 75/30

Substance Abuse History: Denied.

SUBSTANCE ABUSE HISTORY:

Allergies:

No known allergies

Current Medications:

none

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 275089689 Encounter Date: 02/27/2023

Encounter Type: Office Visit

Zoloft 50 MG Oral Tablet; Take 1 tablet orally daily in the morning with food; Qty: 30; Refills: 0

OBJECTIVE:**Vital Signs:**

Height: 65.00 in

Weight: 113.00 lbs

BMI: 18.80

Blood Pressure: 117/70 mmHg

B/P Position: Sitting

Pulse: 70 beats/min

Objective Notes:

MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/ Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language: Intact. Orientation: Ox4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact Concentration/Attention: Intact. Judgment /Insight: Intact

ASSESSMENT:**Diagnosis:**

ICD-10 Codes:

1)F902; Attention-deficit hyperactivity disorder, combined

2)F411; Generalized anxiety disorder

3)F322; Major depressive disorder, single episode, severe

**PROBLEM /
CONDITION:**

Problem/Condition: Established.

Status: Stable

PLAN:**Procedures:**

1) 99214; E&M, moderate problem

2) 90833; Psychotherapy 16-37 min add on code

Procedure Notes:

Michelle presented calm, pleasant and optimistic She was goal directed, engaged in conversation, smiled several times during this visit.

She discontinued Zoloft, "did not like it". However, depression and anxiety has improved.

Requested Adderall 15mg "per pharmacy easier to get".

TREATMENT PLAN:

Switch to Adderall 15mg 2 tablets in the am and 1 tablet in the afternoon. (sent rx for today)

Discontinue Zoloft

Continue Vistaril 10mg TID PRN for anxiety.

She will continue monitoring weight, blood pressure and heart rate at home.

Medications:

Adderall 15 MG Oral Tablet; Take 3 tablets orally daily 2 tablets in the am, 1 tablet in the afternoon (not later than 2pm); Qty: 90; Refills: 0

Care Plan:

The patient gave informed consent for the medication regimen described in the plan of this note.

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 275089689 Encounter Date: 02/27/2023

Encounter Type: Office Visit

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia, tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

For any serious side effects recommended that patient stop the medication immediately.

Patient Instructions:
LEVEL of MDM/
Counseling:

Discussed with the patient HEAT related problem related to psychotropic medications/reinforce hydration
Next Appointment: 3 months. She agreed to call office and schedule appointment sooner if needed.

MODERATE

1. Stable 2 or more illnesses
- 2.a. Reviewed previous note
- 2.b. Reviewed eforce
- 2.c. Reviewed vital signs
3. Prescription drug management

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Counseling/psychotherapy

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Patient shows understanding of counseling.

Psychotherapy conducted for 16 min.

Visit started: 10am

Visit ended: 10:30am

Electronically signed by: Eduardo LaRosa, ARNP

Date: 2/27/2023

_____ Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 268177291 Encounter Date: 10/31/2022

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that s/he understands the limitations of telehealth and consents to proceed with treatment through this mode, zoom.

This is a 35 year old woman with a history of ADHD inattentive type and generalized anxiety disorder. Medication Management visit.

Today Michelle reports "I am feeling well. Everything is going pretty well, super busy with work but I am managing the stress well. I am still trying to figure out to make my business profitable and start hiring people to delegate some things. My medication works well. I am pretty happy where I am".

ADHD: "well controlled".

ANXIETY: "very well controlled, I am not having issues".

COMPLIANCE WITH MEDICATIONS: Yes. SIDE EFFECTS: Denies.

Sleep: "good, my normal at least 7-8 hours per night, no issues with sleep".

Appetite: "good"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

History Of Present Illness: She lives with boyfriend and daughter. Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY: Past Psychiatric History: Yes. ADHD
Psych. Hospitalizations: denies
Past suicidal ideation or behavior: denies
Past violent ideations or behavior: denies
List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies
Florida PMP: overdose risk score 0/999. 7/21/22 Adderall 20mg 75/30.

SUBSTANCE ABUSE HISTORY: Substance Abuse History: Denied.

Allergies: No known allergies

Current Medications: none
Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon no later than 2pm); Qty: 75; Refills: 0
Vistaril 25 MG Oral Capsule; Take 1 capsule orally 2 times a day Take as needed for anxiety; Qty: 60; Refills: 0

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 268177291 Encounter Date: 10/31/2022

Encounter Type: Office Visit

OBJECTIVE:

Vital Signs: Height: 63.00 in
 Weight: 115.00 lbs
 BMI: 20.37
 Blood Pressure: 118/82 mmHg
 B/P Position: Sitting
 Pulse: 69 beats/min

Objective Notes: MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/
 Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought
 Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and
 JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies
 suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language:
 Intact. Orientation: Ox4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory:
 Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact
 Concentration/Attention: Intact. Judgment /Insight: Intact

ASSESSMENT:

Diagnosis: ICD-10 Codes:
 1)F902; Attention-deficit hyperactivity disorder, combined
 2)F411; Generalized anxiety disorder

PROBLEM / Problem/Condition: Established.
 CONDITION: Status: Stable.

PLAN:

Procedures: 1) 99214; E&M, moderate problem
 2) 90833; Psychotherapy 16-37 min add on code

Procedure Notes: Michelle presented calm, pleasant and optimistic. She was goal directed, engaged in conversation and smiled
 several times during this visit.

ADHD symptoms are well controlled with current treatment.

TREATMENT PLAN:

Continue Adderall 20mg 1.5 tablets in the am and 1 tablet in the afternoon. (sent rx for today)

Continue Vistaril 25-50mg BID PRN for anxiety. (did not need refill)

She will continue monitoring weight, blood pressure and heart rate at home.

Medications: Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon
 no later than 2pm); Qty: 75; Refills: 0

Care Plan: The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia,
 tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out
 the day.

Advised the patient to refrain from drug or alcohol while taking medications

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 268177291 Encounter Date: 10/31/2022

Encounter Type: Office Visit

Advised the patient to take the medication with food if GI upset occurs .

For any serious side effects recommended that patient stop the medication immediately.

Patient Instructions:
LEVEL of MDM/
Counseling:

Discussed with the patient HEAT related problem related to psychotropic medications/reinforce hydration
Next Appointment: 3 months. She agreed to call office and schedule appointment sooner if needed.

MODERATE

1. Stable 2 or more illnesses
- 2.a. Reviewed previous note
- 2.b. Reviewed eforce
- 2.c. Reviewed vital signs
3. Prescription drug management

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Counseling/psychotherapy

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Patient shows understanding of counseling.

Visit started: 9:30am

Visit ended: 10am

Electronically signed by: Eduardo LaRosa, ARNP

Date: 10/31/2022

_____ Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 257577902 Encounter Date: 05/03/2022

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that s/he understands the limitations of telehealth and consents to proceed with treatment through this mode, zoom.

This is a 34 year old woman with a history of ADHD inattentive type and generalized anxiety disorder. Medication Management visit.

Today Michelle reports "I am feeling well. Everything is going pretty well. I am still doing real state, but recently my daughter and I started a small business making candles and I love it. It is the first time I am doing something that I am passionate about it. I am doing pretty well".

ADHD: "well controlled since increasing the morning dose of Adderall".

ANXIETY: "very well controlled, no more issues with my ex he was arrested in January, he is out but the stalking stopped".

COMPLIANCE WITH MEDICATIONS: Yes. SIDE EFFECTS: Denies.

Sleep: "good, at least 7-8 hours per night" Taking Melatonin once in while for sleep.

Appetite: "good"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: overdose risk score 0/999 3/24/22, 2/22/22 Adderall 20mg 75/30.

History Of Present Illness:

She lives with boyfriend and daughter.

Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

SUBSTANCE ABUSE HISTORY:

Substance Abuse History: Denied.

Allergies:

No known allergies

Current Medications:

none

Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon no later than 2pm); Qty: 75; Refills: 0

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 257577902 Encounter Date: 05/03/2022

Encounter Type: Office Visit

Vistaril 25 MG Oral Capsule; Take 1 capsule orally 2 times a day Take as needed for anxiety; Qty: 60; Refills: 0

OBJECTIVE:

Vital Signs:

Height: 63.00 in

Weight: 125.00 lbs

BMI: 22.14

Blood Pressure: 115/74 mmHg

B/P Position: Sitting

Pulse: 86 beats/min

Objective Notes:

MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/ Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language: Intact. Orientation: OX4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis:

ICD-10 Codes:

1)F902; Attention-deficit hyperactivity disorder, combined

2)F411; Generalized anxiety disorder

**PROBLEM /
CONDITION:**

Problem/Condition: Established.

Status: Stable.

PLAN:

Procedures:

1) 99214; E&M, moderate problem

2) 90833; Psychotherapy 16-37 min add on code

Procedure Notes:

Michelle presented calm, pleasant and optimistic. ADHD symptoms are well controlled.

TREATMENT PLAN:

Continue Adderall 20mg 1.5 tablets in the am and 1 tablet in the afternoon. Sent rx for today

Continue Vistaril 25-50mg BID PRN for anxiety. did not need refill

She will continue monitoring weight, blood pressure and heart rate at home.

Medications:

Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon no later than 2pm); Qty: 75; Refills: 0

Care Plan:

The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia, tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 257577902 Encounter Date: 05/03/2022

Encounter Type: Office Visit

For any serious side effects recommended that patient stop the medication immediately.

Patient Instructions:
LEVEL of MDM/
Counseling:

Discussed with the patient HEAT related problem related to psychotropic medications/reinforce hydration

Next Appointment: 3 months

MODERATE

1. Stable 2 or more illnesses
- 2.a. Reviewed previous note
- 2.b. Reviewed eforce
- 2.c. Reviewed vital signs
3. Prescription drug management

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Counseling/psychotherapy

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Patient shows understanding of counseling.

Electronically signed by: Eduardo LaRosa, ARNP

Date: 5/3/2022

Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 246564821 Encounter Date: 10/20/2021

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: IN OFFICE VISIT

This is a 34 year old woman with a history of ADHD inattentive type and generalized anxiety disorder. Medication Management visit.

Today Michelle reports "I am feeling pretty well. Everything is pretty well, working full time in real estate and I am doing pretty well. Enjoying making my own schedule. My medication is working pretty well".

ADHD: "well controlled since increasing the morning dose of Adderall".

ANXIETY: "present, but it is well controlled". "a lot better since self employee".

COMPLIANCE WITH MEDICATIONS: Yes. SIDE EFFECTS: Denies.

Sleep: "good, at least 7-8 hours per night" Taking Melatonin once in while.

Appetite: "fine"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: overdose risk score 0/999. 9/23/21 Adderall 20mg 75/30.

History Of Present Illness:

She lives with boyfriend and daughter.

Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

SUBSTANCE ABUSE HISTORY:

Substance Abuse History: Denied.

Allergies:

No known allergies

Current Medications:

none

Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon no later than 2pm); Qty: 75; Refills: 0

Vistaril 50 MG Oral Capsule; Take 1 capsule orally 3 times a day PRN for anxiety and or sleep; Qty: 90; Refills: 0

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 246564821 Encounter Date: 10/20/2021

Encounter Type: Office Visit

OBJECTIVE:

Vital Signs: Height: 63.00 in
 Weight: 125.00 lbs
 BMI: 22.14
 Blood Pressure: 120/83 mmHg
 B/P Position: Sitting
 Pulse: 86 beats/min

Objective Notes: MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/
 Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought
 Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and
 JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies
 suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language:
 Intact. Orientation: Ox4. Time. Place. Person. Situation.
 Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting.
 Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis: ICD-10 Codes:
 1)F902; Attention-deficit hyperactivity disorder, combined
 2)F411; Generalized anxiety disorder

PROBLEM / Problem/Condition: Established.
 CONDITION: Status: Stable.

PLAN:

Procedures: 1) 99214; E&M, moderate problem
 2) 90833; Psychotherapy 16-37 min add on code

Procedure Notes: Michelle presented calm, pleasant and optimistic. ADHD symptoms had improved since titrating Adderall dose.

Agreed to continue Adderall 20mg 1.5 tablets in the am and 1 tablet in the afternoon. Continue Vistaril 50mg
 TID PRN for anxiety

Medications: She will continue monitoring weight, blood pressure and heart rate at home.
 Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon
 no later than 2pm); Qty: 75; Refills: 0
 Vistaril 25 MG Oral Capsule; Take 1 capsule orally 2 times a day Take as needed for anxiety; Qty: 60; Refills:
 0

Care Plan: The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia,
 tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out
 the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 246564821 Encounter Date: 10/20/2021

Encounter Type: Office Visit

Patient Instructions: For any serious side effects recommended that patient stop the medication immediately.

LEVEL of MDM/ Next Appointment: 3 months

Counseling: MODERATE

1. Stable 2 or more illnesses
- 2.a. Reviewed previous note
- 2.b. Reviewed eforcse
- 2.c. Reviewed vital signs
3. Prescription drug management

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Counseling/psychotherapy

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Patient shows understanding of counseling.

Electronically signed by: Eduardo LaRosa, ARNP

Date: 10/20/2021

Date: _____

[Provider]: Eduardo La Rosa, APRN

Center of Revitalizing Psychiatry
2033 Wood Street, Suite 220
Sarasota, FL 34237
Telephone 941-677-3366
Fax 941-677-3367

To:
Carhall Lawfirm
MDA Records Retrieval
Fax: 941-914-9629

Re: Medical Records Request

Invoice Number: 1000173MC

PT: Kohn, Michelle

DATE	SERVICE RENDERED	CHARGE	UNITS	BALANCE
1/16/24	Medical Records Fee	\$25.00	1	\$25.00
	Pages (\$1 per page)	\$48.00	48	48.00

TOTAL AMOUNT DUE \$73.00

**Make payable to: Center Of Revitalizing Psychiatry
or call the office with a credit card number
Tax ID # 043774404**

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 262797900 Encounter Date: 08/01/2022

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that s/he understands the limitations of telehealth and consents to proceed with treatment through this mode, zoom.

This is a 34 year old woman with a history of ADHD inattentive type and generalized anxiety disorder. Medication Management visit.

Today Michelle reports "I am feeling well. I had been sick for a few days with an upper respiratory infection, but emotionally I am doing pretty well. I am still doing real state and working on my business it can be hectic but I am doing well"

ADHD: "well controlled".

ANXIETY: "very well controlled, I am not having issues".

COMPLIANCE WITH MEDICATIONS: Yes. SIDE EFFECTS: Denies.

Sleep: "good, my normal at least 7-8 hours per night, no issues with sleep".

Appetite: "good"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

History Of Present Illness:

She lives with boyfriend and daughter.

Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

Florida PMP: overdose risk score 0/999. 7/21/22 Adderall 20mg 75/30.

SUBSTANCE ABUSE HISTORY:

Substance Abuse History: Denied.

Allergies:

No known allergies

Current Medications:

none

Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon no later than 2pm); Qty: 75; Refills: 0

Vistaril 25 MG Oral Capsule; Take 1 capsule orally 2 times a day Take as needed for anxiety; Qty: 60; Refills: 0

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 262797900 Encounter Date: 08/01/2022

Encounter Type: Office Visit

OBJECTIVE:

Vital Signs: Height: 63.00 in
 Weight: 125.00 lbs
 BMI: 22.14
 Blood Pressure: 123/82 mmHg
 B/P Position: Sitting
 Pulse: 86 beats/min

Objective Notes: MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/
 Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought
 Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and
 JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies
 suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language:
 Intact. Orientation: Ox4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory:
 Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact
 Concentration/Attention: Intact. Judgment /Insight: Intact

ASSESSMENT:

Diagnosis: ICD-10 Codes:
 1)F902; Attention-deficit hyperactivity disorder, combined
 2)F411; Generalized anxiety disorder

PROBLEM / Problem/Condition: Established.
 CONDITION: Status: Stable.

PLAN:

Procedures: 1) 99214; E&M, moderate problem
 2) 90833; Psychotherapy 16-37 min add on code

Procedure Notes: Michelle presented calm, pleasant and optimistic. She was goal directed, engaged in conversation and smiled
 several times during this visit.

ADHD symptoms are well controlled.

TREATMENT PLAN:

Continue Adderall 20mg 1.5 tablets in the am and 1 tablet in the afternoon. (Sent rx for 8/20/2022)

Continue Vistaril 25-50mg BID PRN for anxiety. (did not need refill)

She will continue monitoring weight, blood pressure and heart rate at home.

Medications: Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon
 no later than 2pm); Qty: 75; Refills: 0

Care Plan: The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia,
 tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out
 the day.

Advised the patient to refrain from drug or alcohol while taking medications

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 262797900 Encounter Date: 08/01/2022

Encounter Type: Office Visit

Advised the patient to take the medication with food if GI upset occurs .

For any serious side effects recommended that patient stop the medication immediately.

Patient Instructions:
LEVEL of MDM/
Counseling:

Discussed with the patient HEAT related problem related to psychotropic medications/reinforce hydration

Next Appointment: 3 months

MODERATE

1. Stable 2 or more illnesses
- 2.a. Reviewed previous note
- 2.b. Reviewed eforce
- 2.c. Reviewed vital signs
3. Prescription drug management

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Counseling/psychotherapy

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Patient shows understanding of counseling.

Visit started: 9:30am

Visit ended: 10am

Electronically signed by: Eduardo LaRosa, ARNP

Date: 8/1/2022

Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 273401423 Encounter Date: 01/30/2023

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that she understands the limitations of telehealth and consents to proceed with treatment through this mode, zoom.

This is a 35 year old woman with a history of ADHD inattentive type, depression and generalized anxiety disorder. Medication Management visit.

Today Michelle reports "I am feeling alright. I started the Ritalin but I did not do as well as I do with Adderall. I am still having problems with my ex, which is causing me problems with work, he continues stalking me, it has been hard for me in the last couple months".

DEPRESSION: "Worse": "Feelings of sadness/unhappiness, Irritability or frustration, Loss of interest or pleasure in activities, Indecisiveness, distractibility and decreased concentration, Fatigue, tiredness and loss of energy, Feelings of worthlessness, Feelings of guilt, lacking motivation, crying".

ADHD: "not so good with Ritalin".

ANXIETY: "It has been more present but it is related to work and life has been so busy lately".

COMPLIANCE WITH MEDICATIONS: Yes. SIDE EFFECTS: Denies.

Sleep: "good, my normal at least 7-8 hours per night, no issues with sleep".

Appetite: "good"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

History Of Present Illness: She lives with boyfriend and daughter.
Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY: Past Psychiatric History: Yes. ADHD
Psych. Hospitalizations: denies
Past suicidal ideation or behavior: denies
Past violent ideations or behavior: denies
List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies
Florida PMP: overdose risk score 0/999. 1/9/2023 Methylphenidate 20mg 75/30. 11/8/22 Adderall 20mg 75/30
Substance Abuse History: Denied.

SUBSTANCE ABUSE HISTORY:

Allergies: No known allergies

Current Medications: none

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 273401423 Encounter Date: 01/30/2023

Encounter Type: Office Visit

Vistaril 25 MG Oral Capsule; Take 1 capsule orally 2 times a day Take as needed for anxiety; Qty: 60; Refills: 0

OBJECTIVE:

Objective Notes:

MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/ Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "depressed". Affect: Depressed and anxious. Thought Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language: Intact. Orientation: Ox4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis:

ICD-10 Codes:

- 1)F902; Attention-deficit hyperactivity disorder, combined
- 2)F411; Generalized anxiety disorder
- 3)F322; Major depressive disorder, single episode, severe

**PROBLEM /
CONDITION:**

Problem/Condition: Established.

Status: Worsening.

PLAN:

Procedures:

- 1) 99214; E&M, moderate problem
- 2) 90833; Psychotherapy 16-37 min add on code

Procedure Notes:

Michelle presented depressed and anxious. She was goal directed, engaged in conversation cried during this visit.

We agreed to restart Adderall for ADHD and to start Zoloft for depression and anxiety.

TREATMENT PLAN:

Restart Adderall 20mg 1.5 tablets in the am and 1 tablet in the afternoon. (sent rx for today)

Start Zoloft 25mg daily for depression and anxiety in 2 weeks titrate to 50mg daily.

Taper Vistaril 10mg TID PRN for anxiety.

She will continue monitoring weight, blood pressure and heart rate at home.

Medications:

Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon no later than 2pm); Qty: 75; Refills: 0

hydroOXYzine HCl 10 MG Oral Tablet; Take 1 tablet orally 3 times a day as needed for anxiety and or sleep; Qty: 90; Refills: 0

Zoloft 50 MG Oral Tablet; Take 1 tablet orally daily in the morning with food; Qty: 30; Refills: 0

Care Plan:

The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia, tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out the day.

Advised the patient to refrain from drug or alcohol while taking medications

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 273401423 Encounter Date: 01/30/2023

Encounter Type: Office Visit

Advised the patient to take the medication with food if GI upset occurs .

For any serious side effects recommended that patient stop the medication immediately.

Discussed with the patient HEAT related problem related to psychotropic medications/reinforce hydration

Discussed with the patient the potential side effects of Zoloft including but not limited to GI upset, sexual side effects, sedation, suicidal ideation, dizziness, headache and hypertension.

Next Appointment: 1 month. She agreed to call office and schedule appointment sooner if needed.

Patient Instructions:

LEVEL of MDM/

Counseling:

MODERATE

1. Stable 2 or more illnesses
- 2.a. Reviewed previous note
- 2.b. Reviewed eforce
- 2.c. Reviewed vital signs
3. Prescription drug management

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Counseling/psychotherapy

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Patient shows understanding of counseling.

Psychotherapy conducted for 16 min.

Visit started: 5:10pm

Visit ended: 5:40pm

Electronically signed by: Eduardo LaRosa, ARNP

Date: 1/30/2023

Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 251479171 Encounter Date: 01/20/2022

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that s/he understands the limitations of telehealth and consents to proceed with treatment through this mode, zoom.

This is a 34 year old woman with a history of ADHD inattentive type and generalized anxiety disorder. Medication Management visit.

Today Michelle reports "I am feeling well. Anxiety has increased lately because my ex is stalking me, I had to do a restraining order on him. It has consumed a lot of my time or my energy".

ADHD: "well controlled since increasing the morning dose of Adderall".

ANXIETY: "worse at times, due to my ex".

COMPLIANCE WITH MEDICATIONS: Yes. SIDE EFFECTS: Denies.

Sleep: "good, at least 7-8 hours per night" Taking Melatonin once in while.

Appetite: "fine"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: overdose risk score 0/999. 1/15/21, 12/3/21 Adderall 20mg 75/30.

History Of Present Illness:

She lives with boyfriend and daughter.

Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

Substance Abuse History: Denied.

SUBSTANCE ABUSE HISTORY:

Allergies:

No known allergies

Current Medications:

none

Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon no later than 2pm); Qty: 75; Refills: 0

Vistaril 25 MG Oral Capsule; Take 1 capsule orally 2 times a day Take as needed for anxiety; Qty: 60; Refills: 0

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 251479171 Encounter Date: 01/20/2022

Encounter Type: Office Visit

OBJECTIVE:

Vital Signs: Height: 63.00 in
 Weight: 125.00 lbs
 BMI: 22.14
 Blood Pressure: 119/75 mmHg
 B/P Position: Sitting
 Pulse: 86 beats/min

Objective Notes: MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/
 Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought
 Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and
 JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies
 suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language:
 Intact. Orientation: OX4. Time. Place. Person. Situation.
 Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting.
 Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis: ICD-10 Codes:
 1)F902; Attention-deficit hyperactivity disorder, combined
 2)F411; Generalized anxiety disorder

PROBLEM / Problem/Condition: Established.
 CONDITION: Status: Stable.

PLAN:

Procedures: 1) 99214; E&M, moderate problem
 2) 90833; Psychotherapy 16-37 min add on code

Procedure Notes: Michelle presented calm, pleasant and optimistic. ADHD symptoms are well controlled. Some increase anxiety,
 manageable with Vistaril.

TREATMENT PLAN:

Continue Adderall 20mg 1.5 tablets in the am and 1 tablet in the afternoon. Sent rx for 2/14/22 and 3/16/22
 Continue Vistaril 25-50mg BID PRN for anxiety.

Medications: She will continue monitoring weight, blood pressure and heart rate at home.
 Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon
 no later than 2pm); Qty: 75; Refills: 0
 Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon
 no later than 2pm); Qty: 75; Refills: 0
 Vistaril 25 MG Oral Capsule; Take 1 capsule orally 2 times a day Take as needed for anxiety; Qty: 60; Refills:
 0

Care Plan: The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia,
 tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out
 the day.

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 251479171 Encounter Date: 01/20/2022

Encounter Type: Office Visit

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

Patient Instructions:
LEVEL of MDM/
Counseling:

For any serious side effects recommended that patient stop the medication immediately.

Next Appointment: 3 months

MODERATE

1. Stable 2 or more illnesses
- 2.a. Reviewed previous note
- 2.b. Reviewed eforce
- 2.c. Reviewed vital signs
3. Prescription drug management

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Counseling/psychotherapy

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Patient shows understanding of counseling.

Electronically signed by: Eduardo LaRosa, ARNP

Date: 1/20/2022

Date: _____

[Provider]: Eduardo La Rosa, APRN